



Netramind

STANDARD OPERATING PROCEDURE	
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Title: Good Documentation Practices

1 PURPOSE

This Standard Operating Procedure (SOP) describes the minimum requirements for recording information according to Good Documentation Practices, to ensure that all records and data meet the regulatory principles for ALCOAC++ (Attributable, Legible, Contemporaneous, Original, Accurate, Complete, Consistent, Enduring, Available, and Traceable).

2 SCOPE

This SOP applies to all employees, contractors and service providers involved in generating and documenting records and data including paper and/or electronic version of controlled forms, reports, source data, and any other records or media produced in support of any GxP relevant activity at Netramind.

3 REFERENCES

- [SOP-015 21 CFR Part 11](#)
- [SOP-029 Management of Potential Fraud and Serious Misconduct](#)
- [SOP-006 Deviation Management](#)
- [SOP-014 Computer System Validation](#)
- [SOP-026 GxP CSP Selection and Qualification](#)
- 21 CFR §11: Electronic Records and Electronic Signatures
- 21 CFR §211: General Requirements
- 21 CFR §820.40: Documents Controls
- European Medicines Agency (EMA): EudraLex Volume 4, Part 1: Basic Requirements for Medicinal Products; Annex 11: Computerized Systems
- European Medicines Agency (EMA): Guideline on computerized systems and electronic data in clinical trials
- International Conference on Harmonization (ICH) Q7: Good Manufacturing Practice Guide for Active Pharmaceutical Ingredients; November 2000
- International Conference on Harmonization (ICH) E6 (R3) Good Clinical Practice

4 DEFINITIONS

- 4.1 Accurate—Documentation that is authentic, thorough, and clear.
- 4.2 Approved By—Signature of a qualified individual (such as a supervisor) indicating that the information documented is complete, accurate, and acceptable.
- 4.3 Attributable—Data and documentation that is traceable to a person and date.
- 4.4 Backdating (Pre-dating)—Recording of a signature and/or date that is before the actual date that the document was initialed, signed, or an activity occurred. Backdating does not comply with the ALCOA ++ principle of data being contemporaneous and is therefore unacceptable.



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- 4.5 Certified Copy— A copy (irrespective of the type of media used) of the original record that has been verified (i.e., by a dated signature or by generation through a validated process) to have the same information as the original, including relevant metadata, where applicable.
- 4.6 Contemporaneous—Data and documentation associated with an activity that is recorded as the activity occurs.
- 4.7 Controlled Documents—Documents (i.e., Quality Manual, SOPs, Work Instructions [WIs], Forms [FMs], etc.) required by the Quality Management System (QMS) that are controlled throughout the document lifecycle by identification, storage, protection, version control, retrieval, retention, and disposition.
- 4.8 Data Acquisition Tool (DAT) – A paper or electronic tool designed to collect data and associated metadata from a data originator in a clinical trial according to the protocol and to report the data to the sponsor. The data originator may be a human (e.g., the participant or trial staff), a machine (e.g., wearables and sensors) or a computer system from which the electronic transfer of data from one system to another has been undertaken (e.g., extraction of data from an electronic health record or laboratory system). Examples of DATs include but are not limited to CRFs, interactive response technologies (IRTs), clinical outcome assessments (COAs), including patient-reported outcomes (PROs) and wearable devices, irrespective of the media used.
- 4.9 Data Integrity – Data integrity includes the degree to which data fulfil key criteria of being attributable, legible, contemporaneous, original, accurate, complete, secure and reliable such that data are fit for purpose.
- 4.10 Essential Records – Essential records are the documents and data (and relevant metadata), in any format, associated with a clinical trial that facilitate the ongoing management of the trial and collectively allow the evaluation of the methods used, the factors affecting a trial and the actions taken during the trial conduct to determine the reliability of the trial results produced and the verification that the trial was conducted in accordance with GCP and applicable regulatory requirements.
- 4.11 Falsification—Intentionally recording incorrect or inaccurate data.
- 4.12 GxP Records— Records associated with the development, manufacture, packaging, testing, marketing, and distribution of the active substance (i.e., API), pharmaceutical product (i.e., drug product), or medical device that are used for clinical and commercial purposes. This also includes registration and regulatory records for both clinical and commercial products.
- 4.13 Indelible—Cannot be washed away, erased, easily removed, or smudged. Ink that runs when wet, as with felt tip pens, is not indelible.
- 4.14 Legible—Data and documentation must be clear enough to read.



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- 4.15 Metadata— The contextual information required to understand a given data element. Metadata is structured information that describes, explains or otherwise makes it easier to retrieve, use or manage data. For the purpose of this guideline, relevant metadata are those needed to allow the appropriate evaluation of the trial conduct.
- 4.16 Original record/data— Data as the file or format in which it was originally generated, preserving the integrity (accuracy, completeness, content and meaning) of the record, e.g., original paper record of manual observation, or electronic raw data file from a computerized system. (Source: Data Integrity Definitions and Expectations (Revision 1), Jan, 2015, MHRA)
- 4.17 Performed By—Initials or signature of the person executing an operation or task.
- 4.18 Postdating— Recording of a signature and date with a date later than the execution date. Postdating does not comply with the ALCOAC++ principle of data being contemporaneous and is therefore unacceptable.
- 4.19 Raw Data— Factual recorded information obtained as a result of original observations or activities, which has not been copied or transcribed. This term is synonymous with original data.
- 4.20 Reviewer—An individual who checks over a document for accuracy to confirm that all required supporting data, references, signatures, etc. are present.
- 4.21 Signature —A unique mark, symbol or entry executed, adopted or authorized by an individual, in accordance with applicable regulatory requirements and/or practice to show expression of will and allow authentication of the signatory (i.e., establish a high degree of certainty that a record was signed by the claimed signatory). A signature may be physical or electronic.
- 4.22 Source Records— Original documents or data (which includes relevant metadata) or certified copies of the original documents or data, irrespective of the media used. This may include trial participants' medical/health records/notes/charts; data provided/entered by trial participants (e.g., electronic patient-reported outcomes (ePROs)); healthcare professionals' records from pharmacies, laboratories and other facilities involved in the clinical trial; and data from automated instruments, such as wearables and sensors.
- 4.23 True Copy—An exact copy of an original record, which may be retained in the same or different format in which it was originally generated, e.g., a paper copy of a paper record, an electronic scan of a paper record, or a paper record of electronically generated data.
- 4.24 Verified By—Initials or signature of a person examining a task, document, or record to confirm its accuracy and completeness.

5 RESPONSIBILITIES

- 5.1 All Netramind employees, contractors and service providers (SPs)



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- 5.1.1 Completes, reviews, and/or approves handwritten, printed, and electronic records in compliance with standard procedures, plans and GDP.

6 PROCEDURE

6.1 Employee Signatures

- 6.1.1 Personnel must be uniquely identifiable in all records via name, signature, or system-based authentication.

6.2 Glossary of Terms

- 6.2.1 Definitions and abbreviations used across documents must be consistent and defined within the documents in which they are being used.
- 6.2.2 If Netramind maintains a Glossary, terms and definitions must be current per regulatory guidance, regulations and industry standards.

6.3 General Data Recording Guidelines on Paper Source

- 6.3.1 All data must be attributable, legible, contemporaneous, original, accurate, complete, consistent, enduring, available, and traceable (ALCOAC++).
- 6.3.2 An individual cannot perform and approve or verify the same task.
- 6.3.3 All information and data must be recorded in real time (i.e., when the activity/ task was performed).
- 6.3.4 All manual data entries must be made by the employee who performed the task. If the original member is not able to make entries and another member has the document on behalf of the original member, a note should be clearly indicated with accompanying initials and date.
- 6.3.5 All signatures/ initials must be accompanied by a date. Assigning one date for all signatures/ initials on a single page is not acceptable.
- 6.3.6 Entries must be made immediately after completion of a step directly onto the original written document. If data or other information were omitted and recorded later, employees must:
- 6.3.6.1 Add a justification explaining why the data was added later,
- 6.3.6.2 Reference supporting evidence, if any, that authenticates the late entry, and
- 6.3.6.3 Add their initials and the date on which the late entry was added.
- 6.3.7 Backdating and postdating are not permitted. If an error is discovered, use the current date on which the correction is made. Clearly indicate who is making the correction (initials), what occurred, how and where the information was recovered, and why (providing a clearly written, concise, easily understood, and scientifically sound explanation). A manager's signature may be needed to verify the correction.



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- 6.3.8 Unless specifically required by a protocol or SOP, there should only be one set of original study data (for a clinical study).
- 6.4 Guidelines for data recording using Data Acquisition Tools
 - 6.4.1 Data acquisition tools (DATs) should be documented as being fit for purpose.
 - 6.4.1.1 For DATs developed by Netramind, respective software development and testing procedures must be adhered to, and then the DAT must be validated in accordance with **SOP-014 Computer System Validation**.
 - 6.4.1.2 For vendor-supplied DATs, those vendors must first be qualified per **SOP-026 CSP Selection, Qualification, and Oversight**.
 - a. An appropriate risk-based validation approach must be undertaken for vendor-supplied DATs as per **SOP-014 Computer System Validation**.
 - 6.4.2 Procedures specific to each DAT must be developed which govern their study-specific configuration and testing, defined user roles and periodic reviews of user access, requirement to define a risk-based audit trail review approach, and appropriate use of the DAT.
 - 6.4.2.1 If the data captured in the DAT can be unblinding, a blind maintenance plan must be created.
 - 6.4.3 DATs must:
 - 6.4.3.1 Have audit trail functionality that is exportable and legible.
 - 6.4.3.2 Only capture data and information required by the protocol.
 - 6.4.3.3 Be validated and then configured for study-specific use prior to the start of the study.
 - 6.4.3.4 Have a training program for all user types to enable appropriate use of the DAT.
 - 6.4.4 The above can be delegated to a third-party; however, the third-party must be able to meet all requirements of Netramind for use of DATs.
- 6.5 Data Correction
 - 6.5.1 The use of correction materials such as correction fluid or tape that obliterates original information is not permitted. The original entry must remain legible or visible.
 - 6.5.2 A comment explaining the reason for the correction must be included in the Comment section or elsewhere on the same page as the correction.
 - 6.5.3 If corrections are made after the date of the original data entry, the Department Manager reviews and approves justification for the changes. Document an explanation about the correction.



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- 6.5.4 If an individual fails to sign and date information on an entry and a correction is made at a later date, sign and date the entry on the date the correction is made and indicate the original entry date. Include an explanation for the correction.
- 6.5.5 Corrections must be made by the individual who performed the documentation, with the following exceptions:
 - 6.5.5.1 Retrievable information can be corrected by anyone, such as entries made on summaries from raw test data.
 - 6.5.5.2 A correction to a document made by an individual who is no longer employed by Netramind or is not available can be performed by management.
- 6.5.6 If an original document must be rewritten due to damage, attach the original document. Provide an explanation for the rewritten document.
- 6.5.7 Corrections made after a record is approved require an explanation and must be approved by the original approvers of the record or their equivalent role.
- 6.6 Concealing Private/ Confidential Information on Source Documents
 - 6.6.1 Personal/identifiable information must be appropriately protected or redacted in accordance with data privacy requirements.
- 6.7 Clarifying Information
 - 6.7.1 Clarification is required when information on a document is illegible, not clear, or can lead to misinterpretation.
 - 6.7.2 The individual entering clarification details must initial and date the document.
- 6.8 Completeness of Records
 - 6.8.1 When a document or record requires an entry to provide evidence of activity completion and the activity was performed but not documented, include an explanation of why there was an omission.
- 6.9 Transcribing Data
 - 6.9.1 Where data is transcribed, the transcription must be accurate, attributable, and verified. Original records must be retained or referenced to maintain traceability.
- 6.10 Recreation of Lost Records
 - 6.10.1 All instances of original lost records must be documented as a deviation per **SOP-006, Deviation Management**, regardless of whether content could be recreated from other sources.
- 6.11 Handling Fraud in Documentation



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6.11.1 Fraudulent documentation practices are prohibited and should result in disciplinary action and/or termination of employment. **SOP-029 Management of Potential Fraud and Serious Misconduct** provides further details.

6.11.2 Fraudulent documentation practices include, but are not limited to:

- Altering data to meet acceptance criteria.
- Unqualified staff performing a task.
- Obscuring original data
- Forging signatures
- Backdating data entries
- Back-entering data with no traceability
- Disposing of unused data
- Entering data results when testing was not performed.
- Signing and entering dates and times prior to or after activities have been performed.
- Destroying original data without approval
- Verifying a step, task, calculation, or other entry without observation



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7 REVISION HISTORY

Version Number	Date	Summary of Change(s)
01	01-MAY-2026	New Document
[X.0]	[DD-MMM-YYYY]	[Insert a summary of the changes]



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SIGNATURE PAGE

[ADD ADDITIONAL ROWS AS NEEDED]

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